
CONSULT FOR AFRICA

Osteon Clinics: Positioning Discovery

A short intake to ground the brand strategy in your real numbers

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Why this exists

Our positioning recommendation (lead as the complex-case and revision authority) rests on a few assumptions about your practice that only you can confirm. This questionnaire tests them. It should take about 20 minutes. Rough numbers and honest estimates are fine; we are looking for the shape of the practice, not audited accounts. Where you would rather talk something through than write it, just note "discuss" and we will cover it in the working session.

A plain answer to any question of "I don't track this" is itself useful, because it usually points to something worth starting to measure.

1. Case mix (the most important section)

This tells us whether the complex-case position is a genuine repositioning or simply naming what you already do.

1.1 Roughly how many surgical cases do you do in a typical month, across all sites?

1.2 Of those, what is the rough split (in %) between:

- Primary hip/knee replacement
- Revision arthroplasty (redoing a previous replacement)
- Bone and joint infection cases
- Complex trauma and limb reconstruction
- Other (please describe)

1.3 How often do other doctors or surgeons send you a case they could not manage themselves, or that had gone wrong? Never / occasionally / regularly / it is already a meaningful share.

1.4 Have you operated on patients whose original surgery was done abroad (India, Turkey, elsewhere) and came back with a problem? Roughly how often?

1.5 What is the one type of case you are known for among other doctors, in your own words?

2. Who pays, and how much

2.1 For a primary knee or hip replacement at Osteon, what does the patient typically pay, all-in? (A range is fine.)

2.2 For a complex or revision case, what is the typical range?

2.3 Rough split of how your cases are funded:

- Self-pay (patient's own money)
- Diaspora-funded (family abroad paying)
- HMO / insurance
- Corporate / employer
- Other

2.4 Do any HMOs currently cover joint replacement with you? If so, which, and how well do they actually pay?

2.5 In your sense, how price-sensitive are your self-pay patients? Do they shop on price, or on who they trust?

3. Where patients come from now



3.1 Roughly, how does a new patient find you today? Give your best % split:

- Referral from another doctor
- Referral from a physiotherapist or other allied professional
- Word of mouth from a former patient
- Found you online (website, Google, social)
- Through one of the hospitals you visit (Cedarcrest, Diamed, others)
- Other

3.2 Who are your top three individual referrers (people or clinics that send you patients), if you can name them?

3.3 Comparing your Amuwo Odofin base with your visiting work at Cedarcrest, Diamed and elsewhere: where do more of your patients (and your better-paying patients) actually come from?

3.4 Do you currently do anything deliberate to stay front-of-mind with referring doctors? (Thank-you notes, updates on their patients, teaching, WhatsApp, nothing yet.)

4. The sites and the visiting model

4.1 List the places you currently operate or consult, and roughly how many days a month you spend at each.

4.2 At Osteon Amuwo Odofin, what can you do on site (theatre, imaging, inpatient beds, physiotherapy), and what do patients have to go elsewhere for?

4.3 You own Osteon, and we are treating you (Dr Bola) as the primary brand and Osteon as the secondary, supporting institution. Where do you want Osteon to be in five years: your personal practice vehicle, or a standalone institution that could carry other surgeons, services and sites and grow beyond you? This shapes how independent we make the Osteon brand.

4.4 Is there any appetite (now or later) to add a formal presence on the Island, or to bring a diaspora surgeon in for batch complex lists? Just a yes / no / maybe.

5. You, and the brand today

5.1 In one or two sentences, how do you currently describe what you do when a non-medical person asks?

5.2 What do you most want to be known for in five years?

5.3 Is there any part of your work you find most meaningful, or any type of patient you most want more of?

5.4 Are you comfortable being the face of the brand (your name, your story, video, teaching, thought leadership)? Very / somewhat / would rather lead with the clinic.

5.5 Anything you would not want said or done in your name (claims, tone, comparisons to named competitors)?

6. Practicalities

6.1 Do you have your case numbers, outcomes or complication rates recorded anywhere? Even informally? (Strong outcome data is the single most persuasive proof point we can put in front of patients.)

6.2 Do you have before/after imaging, patient consent for stories, or testimonials we could draw on?

6.3 Who controls the current website and social accounts, and are you happy for us to work on them?



6.4 What does success from this engagement look like to you, concretely, twelve months out?

Return this however is easiest (typed, voice notes, or a call). We will read it before the working session and come back with the positioning locked and a build plan.